DESIGN SAMPLE, NOT CHAPTER TEXT

Emergency Nursing Fundamentals

A Compact Illustrated Guide to Rapid Assessment, Priorities and Procedures for Nursing, Paramedic, EMT and Allied Health Students

MARK GERALD SIMON

Copyright © 2026 Mark Gerald Simon. All rights reserved.

DESIGN SAMPLE. The wording on these pages is placeholder text written only to show the layout. It is not chapter text, has not been fact-checked, and must not be used for care.

This book is for education only. It is not a substitute for training, local protocol or medical direction. In an emergency, contact local emergency services.

Contents

Part 1 The Emergency Approach: Safety, Triage and Assessment 5

1.1 Scene Safety and Responder Wellbeing 6

1.2 Rapid Triage 7

Part 2 Airway and Breathing Emergencies 17

Reference Glossary 19

PART 1

The Emergency Approach: Safety, Triage and Assessment

IN THIS PART

1. Explain why scene safety comes before patient care.
2. Sort patients by how long they can safely wait.
3. Describe a short, repeatable assessment sequence.

Every emergency starts the same way: someone arrives, takes in a scene they did not choose, and has seconds to decide what matters first. This Part gives you a calm order of thinking. It begins with the ground you are standing on, moves to the patient who cannot wait, and ends with an assessment you can repeat under pressure.

1.2 Rapid Triage

Triage is not a judgment about who deserves care. It is a way of deciding who cannot wait. A patient who walks in talking, holding a swollen ankle, can sit for a while. A patient who looks

calm but is breathing far too fast cannot, and the person at the desk has perhaps thirty seconds to notice.

WHAT IT IS. Triage sorts patients into groups by urgency so that limited people and equipment reach the sickest first. The groups are a tool for the moment, and a patient's group can change as you watch.

WHY IT MATTERS. The quiet emergency is easily missed behind the loud inconvenience. A short, repeatable method protects the patient who does not complain.

RED FLAGS

Placeholder wording: difficulty speaking in full sentences, a gray or blue tint to the lips, confusion that is new, or a patient who suddenly stops responding. Call for help at once.

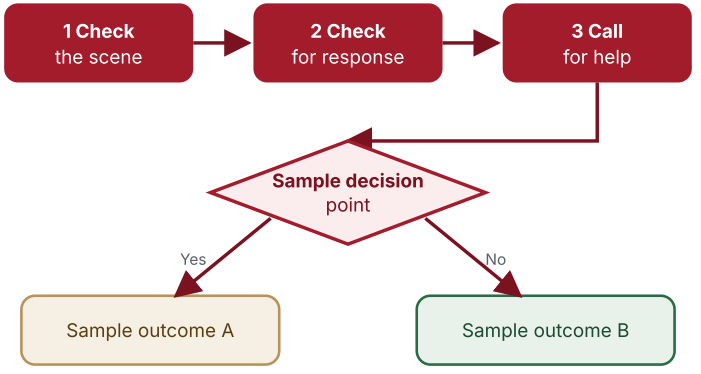


Figure 1. Sample figure slot: a flowchart drawn as vector art. Real figures show one idea and keep every label large enough to read at this page size.

SCOPE NOTE

What you may do depends on your training level and local protocol. If a step is outside your role, say so and call someone who is trained.

1.3 Checking the Scene

Before you kneel beside the patient, look at the scene once more. Check for traffic, spilled fluid, loose electrical equipment and anyone who may still be a danger. Your first duty is to arrive able to help, because an injured responder is a second patient and takes a rescuer away from the first.

PROCEDURE: MAKE THE SCENE SAFE

Purpose: to stand where you can help without becoming a casualty. Scope: follows your training level and local protocol.

1. Stop at the edge of the scene and look in every direction.
2. Put on gloves and any protection your role requires.
3. Name each hazard aloud, even if you are alone.
4. Remove the hazard only if it is safe and within your role; otherwise wait for help.
5. Tell the person you are there to help and ask them not to move.

SAFETY

Never enter a scene you believe is unsafe. A second casualty helps no one. Placeholder wording for design only.

STOP AND CALL FOR HELP IF the hazard is still present or you are unsure it has gone.

Sample comparison table (placeholder wording)

Group	What you see	What it means
Cannot wait	Struggling to breathe	Help right now
Can wait briefly	Stable but in pain	Recheck often
Can wait	Walking and talking	Reassess later

REMEMBER

Look, listen, then act. One short sentence to hold onto.

ILLUSTRATIVE CASE: NOT A REAL PATIENT

A student on placement notices a man in the waiting area who has stopped speaking in full sentences. Placeholder wording to show how the case box looks.

CARE PRIORITIES

First, make the scene safe. Second, find the patient who cannot wait. Third, assess in a fixed order and recheck. Placeholder wording to show the summary box.

QUICK CHECK

1. Why does scene safety come first?
2. Name one sign that a patient cannot wait.
3. What should you do if a step is outside your role?

REFERENCE

Glossary

A

Airway. The passage that carries air to the lungs. Placeholder wording.

Assessment. A fixed order of checks used to find what is wrong first. Placeholder wording.

B

Breathing. The movement of air in and out of the lungs. Placeholder wording.

Bleeding, severe. Blood loss that cannot be allowed to continue. Placeholder wording.

T

Triage. Sorting patients by how long they can safely wait. Placeholder wording.